FLINTRIDGE CLINIC / PHARMACY COMPLIANCE REVIEW

Pinehollow Regional Office Pharmacy — acquisition credit committee memo

To Fiona Okonkwo, Chair; Credit Committee **From** Genevieve Eberhardt, Pharmacist **For** Kwame Boateng • Wednesday, Aug. 19, 2026

REPLACED RISK ASSESSMENT	DECISION ANCHOR	SCOPE
Preliminary "acceptable" → pharmacy / operating risk: DEFER on current record.	**0.85x stressed DSCR** $270,000 ÷ $318,000 annual debt service	Pennsylvania pharmacy requirements as of **August 15, 2026**

Credit case: headline is not repayment capacity

Established transaction facts. Flintridge proposes an **asset acquisition** of prescription files, saleable inventory, fixtures and the leasehold position, closing **October 18, 2026**: **$2.85m** purchase price, funded by a requested **$2.25m** seven-year term loan plus **$600,000** buyer equity (sources = price).

Economics. Seller-reported adjusted EBITDA is **$620,000**, but includes owner add-backs and assumptions underwriting has not accepted. The lender case is **$355,000** before the Payer A reset (**1.12x** = $355,000 ÷ $318,000) and **$270,000** after its estimated effect: **0.85x stressed DSCR**. Below 1.00x means modeled cash flow does not cover **$318,000** annual debt service. Management's **$145,000** purchasing/staffing synergies have no executed vendor terms or staffing support and are excluded.

Calculated implication. The preliminary label relied on profitable operations; review now shows thin normalized coverage and cash flow below debt service immediately after closing, before separately estimated year-end reconciliation exposure of **$180,000–$310,000**.

Volume and payer: structural loss, concentrated margin

Established claims trend. Scripts: **78,400 FY2024**; **72,900 FY2025**; **64,800 FY2026 annualized**. Revenue: **$12.4m**; **$11.7m**; **$10.6m FY2026 annualized**. The 13,600-script decline (17.3%) is attributed in the claims review to **9,700** after a referring group relocated (71.3%), **2,100** after a payer-network exclusion, and **1,800** general attrition. Seller representation: temporary technician vacancies caused the decline. No executed referral replacement or documented recovery plan supports a rebound; the evidenced loss is substantially structural.

Payer dependence and spread. Payer A is **47%** of scripts and **52%** of gross reimbursement; B and C add 19%/16% and 13%/11%, respectively—top three = **79%** of both scripts and gross reimbursement. That makes a small number of network/repricing decisions earnings-critical. Gross reimbursement per prescription fell **$124.60 → $116.20**; the distinct amount remaining after ingredient cost, dispensing-related fees and known concessions fell **$12.40 → $8.10**.

Near-term reset. Payer A notified seller that, if mix holds, **November 1, 2026** reduces that remaining amount by **$2.80** on its scripts—about **$85,300** annualized (64,800 × 47% × $2.80), just after October 18 closing. This reset and the $180,000–$310,000 reconciliation range are distinct exposures. Buyer payer enrollment/network continuity is unconfirmed and does not transfer with assets.

Pennsylvania operation: authority and controlled substances

Established fact / unresolved application. A Pennsylvania State Board of Pharmacy consent order naming seller pharmacist-in-charge **Helena Kilgore** runs **Feb. 12, 2025–Feb. 11, 2027**, so is open at closing. It arose from controlled-substance recordkeeping and inventory-reconciliation failures and requires quarterly controlled-substance audits, documented corrective action and enhanced pharmacist supervision. It does *not* name Flintridge; asset purchase does not itself make it Flintridge's liability. But the plan relies on Helena through transition, creating supervision and opening-readiness risk until authoritative confirmation addresses application to her, the existing permit and the transition plan.

Before buyer operation, unresolved. A buyer pharmacy permit/ownership authorization and an eligible buyer pharmacist-in-charge must be in place; neither follows the assets. DEA authority is registrant- and location-specific, not transferable. The file lacks the ownership-change filing, buyer permit, PIC readiness, DEA/control protocol and documented closing inventory/lawful controlled-substance transfer treatment. Prescription and controlled-substance records must remain accessible and retained; prescription-file handling must be lawful and minimum-necessary, with patient choice, communication and continuity preserved.

Occupancy and asset-transfer dependency

Established lease fact. The location lease runs through **Sept. 30, 2030** at **$18,500/month** base rent. Assignment or subletting requires the landlord's **prior written consent**; tenant has **no unilateral assignment right**. Seller reports the landlord is supportive—an unverified representation, not consent. There is no signed consent, lease amendment or replacement lease.

Calculated implication. Without documented occupancy control, the licensed location, opening, prescription-file transition and payer participation cannot be assumed uninterrupted. The seven-year loan also extends beyond the current 2030 lease expiry. Inventory value/transferability remains dependent on excluding or properly handling expired, recalled, damaged, unsaleable and return-to-vendor stock.

Why this changes the label. The reported EBITDA and seller explanations do not cure the modeled **0.85x stressed DSCR**, structural script loss, payer concentration/repricing, open-order transition reliance, or lack of assignable occupancy. These are not routine post-closing clean-up items; each can interrupt the legally authorized, payer-participating operation that produces repayment cash flow.

DECISION — DEFER THE $2.25 MILLION LOAN REQUEST ON THE CURRENT RECORD.